

Treating Physician's Detailed Notes

Date: August 15, 2025

Physician: Dr. Aisha Al Qubaisi, Consultant Orthopaedic Surgeon

Facility: CityMed Hospital, Al Safa, Dubai, UAE

Subjective:

Patient Leila Al Mansouri, 34-year-old female, presents with persistent left knee pain for the past 6 weeks. Pain is described as dull, occasionally sharp with movement, and rated 7/10. Reports progressive swelling and stiffness, particularly in the morning and after long periods of standing. Denies trauma or recent injury. OTC NSAIDs provide partial, short-term relief.

Objective:

- **Inspection:** Visible swelling around the left knee joint. Mild erythema.
- **Palpation:** Tenderness along medial joint line, mild warmth noted.
Range of Motion: Limited flexion to 90° due to pain, full extension possible but uncomfortable.
- **Special Tests:** McMurray test elicited pain medially; Lachman's test negative.
- **Vital Signs:** Stable, afebrile.

Assessment:

- Suspected internal derangement of the left knee (possible meniscal tear vs. cartilage injury).
Differential diagnosis includes ligamentous injury or early degenerative changes.

Plan:

- Order MRI of the left knee (CPT 73721) to assess meniscal integrity, cartilage damage, and exclude other intra-articular pathology.
- Continue conservative management: NSAIDs as needed, rest, and avoidance of weight-bearing exercise until further imaging is available.
- Follow-up after MRI results to determine definitive management (physiotherapy vs. surgical intervention if indicated).

Medical Necessity Statement:

Given the patient's persistent symptoms, functional limitation, and physical examination findings, advanced imaging via MRI is medically necessary to guide treatment decisions and prevent further deterioration of the joint.

Signature:

Dr. Aisha Al Qubaisi

Consultant Orthopaedic Surgeon

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